A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 May 2026

Dr Adrian Dragovic
Medical One Moonee Ponds

Dear Dr Dragovic,

Re: Dr Tessa Potezny
DOB: 18/05/1994 Phone: 0438 602 372
Address: 9 / 52 Mirams Avenue, ASCOT VALE, VIC, 3032
Email: tessa.potezny@gmail.com

Summary

- **Normal gastroscopy and colonoscopy.**
- **Histology confirmed no evidence of chronic inflammatory bowel disease or architectural changes.**
- **Improvement in functional bowel symptoms with lifestyle modification and PRN Movicol.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Tessa following her gastroscopy and colonoscopy. You will be pleased to know that the investigations were essentially normal.

While I noted some mild rectal erythema during the procedure, the biopsies have returned as entirely normal. There were a few scattered acute inflammatory cells, but critically, there was no evidence of chronic inflammatory cells or architectural changes that would suggest a diagnosis of inflammatory bowel disease. I suspect the mild macroscopic findings were simply related to the bowel preparation.

Tessa is feeling significantly better. As a colleague and a PhD candidate, she acknowledged a degree of underlying anxiety regarding the etiology of her symptoms, and the normal results have provided substantial reassurance. She has been successfully managing her bowel habit by modifying her morning routine—specifically by allowing time for breakfast and coffee before engaging in high-intensity exercise—and using Movicol approximately three times a week as needed for harder stools.

Impression and Plan

At this stage, Tessa's symptoms are manageable and do not require escalated therapy. We discussed the potential role of a bowel-focused pelvic floor physiotherapist in the future, particularly given her high level of athletic activity, to assist with core muscle relaxation and defaecatory mechanics. However, she feels this is not necessary at present given her symptomatic improvement. I have advised her that no routine surveillance is required and have encouraged her to reach out should she experience any new red-flag symptoms. I will review her only on an as-needed basis. Thank you for referring her!

Yours sincerely,

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Gastroenterologist

Investigation Results:

****Not Authorised!** 15 May 2026, 08:51:12:130**

POTEZNY, TESSA MARIE : SVP 926690

DOB: 18/05/1994 Age: 31 years

71 Victoria St, Brunswick East 3057

Results from 11/04/2026 ..... Episode# 26P275612 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009424 **CLINICAL NOTES** Gas = N. Duo = N. Oesophagus = N. TI = N. Random = N. Rectum = Subtle inflammation =? Unclear aetiology. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophagus bx". Two mucosal fragments measuring 2-3 mm. A1. 2. Specimen labelled "Gastric bx". Two mucosal fragments measuring 3-4 mm. A1. 3. Specimen labelled "Duodenum bx". Three mucosal fragments measuring 2-7 mm. A1. 4. Specimen labelled "TI bx". Two mucosal fragments measuring 1-5 mm. A1. 5. Specimen labelled "Random Colon bx". Two mucosal fragments measuring 1-3 mm. A1. 6. Specimen labelled "Rectal bx". Two mucosal fragments measuring 1-4 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The biopsy shows two fragments of squamous epithelium with minimal attached stroma. No active inflammation, organisms, dysplasia or malignancy is identified. 2. The biopsy shows three fragments of nonspecialised gastric mucosa with mild convolution of gastric pits, splaying of muscularis mucosae and mucin depletion, in keeping with mild reactive gastropathy. No active inflammation or Helicobacter pylori like organisms are seen. There is no evidence of intestinal metaplasia, dysplasia or malignancy. 3-4. These biopsies show fragments of small intestinal mucosa with villous architecture. There is no villous blunting or increased intraepithelial lymphocytes. No active inflammation is seen. There is no evidence of ulceration, organisms, granuloma formation, dysplasia or malignancy. 5. The biopsy shows two fragments of colonic mucosa with preserved crypt architecture. The lamina propria contains a normal complement of chronic inflammatory cells. No active inflammation is seen. There is no subepithelial collagen plate thickening or increased intraepithelial lymphocytes. There is no evidence of ulceration, granuloma formation, organisms, dysplasia or malignancy. 6. The biopsy shows two fragments of colonic mucosa with preserved crypt architecture. The lamina propria appears to contain a relatively normal complement of chronic inflammatory cells. Very rare neutrophils are seen in the lamina propria and focally within surface epithelium. No crypt abscesses or ulceration is seen. No organisms, granuloma formation, dysplasia or malignancy is identified. There is no thickening of the subepithelial collagen plate or increased intraepithelial lymphocytes. **PATHOLOGIST'S OPINION** **1. Oesophagus biopsy: Within normal limits. 2. Gastric bx: Mild reactive gastropathy. 3. Duodenum bx: Within normal limits. 4. TI bx: Within normal limits. 5. Random colon bx: Within normal limits. 6. Rectal bx: Very focal active colitis.** Pathologist: Dr Joan Wang

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